A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 April 2026

Dr Glen Guerra
Melbourne Colorectal Specialists

Dear Dr Guerra,

Re: Mrs Onella Stagoll
DOB: 1946-03-19
Address: 256 Mckean Street, North Fitzroy, Vic, 3068

I would be grateful if you could see Onella, a long standing patient of mine.

She has had recurrent uncomplicated diverticulitis, but unfortunately had perforation during her trip to Hobart. She had a hartmans performed by, what sounds like a general surgeon rather than a colorectal one. Her stoma is now situated within a fold on her abdominal wall - so she cant access it. This admission was complicated by a collection for which I am repeating a CT for this week - SVPHM. Her daughter (a nurse) has forwarded all of the hobart scans to SVPHM.

I would be grateful if you could see her for

1. Stoma - when or if it is apropriate to have it repositioned/reversed
2. What to do about the collections.

Chamara

Summary

- **Recent complex admission for perforated diverticulitis, complicated by rapid AF, AKI, collection and stoma formation.**
- **Improving inflammatory markers with a current CRP of 70.**
- **Hypomagnesaemia (0.61 mmol/L) in the context of atrial fibrillation.**
- **Referrals initiated for colorectal surgery and cardiology.**

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
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- **Action requested if possible: repeat blood tests (FBE, UEC, CRP, Magnesium) in one week to monitor renal function and electrolyte replacement.**

I had the pleasure of conducting a telehealth review with Onella and her daughter Lilly following her recent hospitalisation in Hobart for a diverticular perforation and subsequent stoma formation. I have reviewed her recent pathology and am awaiting the transfer of her operative reports and imaging from Tasmania.

Clinically, Onella is currently stable. She denies any acute abdominal pain, fevers, or sweats. Her primary gastrointestinal complaints at present are bloating and frequent eructation. She is not experiencing symptoms of acid regurgitation or heartburn. Consequently, I have advised her not to increase her pantoprazole dose, as it is unlikely to alleviate the burping. She continues to use ondansetron sparingly for nausea.

Her recent biochemistry from Melbourne Pathology shows a downward trend in her C-reactive protein, decreasing from 84 to 70. Her creatinine is 97 µmol/L with an eGFR of 48 mL/min/1.73m², which will require ongoing monitoring. Most notably, her magnesium level is low at 0.61 mmol/L. Given her history of atrial fibrillation, it is clinically imperative to correct this to mitigate the risk of arrhythmia.

Impression and Plan

I have requested that Onella temporarily increase her magnesium supplementation to eight tablets daily for the coming week to address the hypomagnesaemia. I have also provided a repeat prescription for ondansetron. She remains on a course of ciprofloxacin and metronidazole as previously prescribed.

Regarding her specialist follow-up, I am referring her to Mr Glenn Guerra for colorectal surgical review. I have also recommended a cardiology review, suggesting Dr Andrew Burns, Dr Elizabeth Parratz, or Dr Belinda Smith, and I will assist in determining who can see her earliest. I will review her CT imaging remotely while I am overseas and will contact Onella directly if there are any findings that necessitate an immediate change in management. I would appreciate it if you could facilitate repeat blood pathology in one week's time to monitor her progress from the perspective of the CRP and magnesium - I hope this is ok with you.

Yours sincerely,

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